SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Topotecan by infusion · small-cell lung cancer

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Relapsed small-cell lung cancer · NCCP 00311 · version 4 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This treatment aims to slow or shrink small-cell lung cancer that has returned after treatment, when repeating the first treatment is not appropriate. Benefit varies; your team explains the aim for you.

Schedule
Topotecan is given by infusion on Days 1–5 of a 21-day cycle. Up to 6 cycles are planned, with earlier changes if treatment stops helping or side effects become unacceptable.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Topotecan infusion

TOPOISOMERASE-I INHIBITOR CHEMOTHERAPY

- ☐ Low blood counts, infection and bleeding
- ☐ Fatigue and anaemia
- ☐ Nausea, vomiting, diarrhoea and mouth soreness
- ☐ Hair loss
- ☐ Allergic reactions
- ☐ Fertility and pregnancy effects
- ☐ Rare serious lung inflammation
- ☐ Rare severe bowel inflammation or perforation

Discuss treatment aim, alternatives including symptom-focused care, common and serious risks, fertility, possible delays or stopping, and what matters most to you.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00311 v4.

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